



NCI Protocol #: 10323

Patient ID:

Date:

1 of 4

Patient Name:

Date Collected:

Biopsy Site: Lung

Sample ID:

Telephone:

Tumor Content (%): 10_19

Patient DOB:

Referring Physician:

Primary Diagnosis: Lung adenocarcinoma

MoCha ID:

Fax:

Preanalytics to include tumor enrichment as required is performed by Van Andel Research Institute; Grand Rapids; MI 49503

Table of Contents

Page

Variant Details

1

Biomarker Descriptions

2

Report Highlights

0 Relevant Biomarkers

0 Therapies Available

0 Clinical Trials

Relevant Non-Small Cell Lung Cancer Findings

Gene	Finding	Gene	Finding
ALK	None detected	NTRK1	None detected
BRAF	None detected	NTRK2	None detected
EGFR	None detected	NTRK3	None detected
ERBB2	None detected	RET	None detected
KRAS	None detected	ROS1	None detected
MET	None detected		

Relevant Biomarkers

No biomarkers associated with relevant evidence found in this sample

Prevalent cancer biomarkers without relevant evidence based on included data sources

TP53 R273C

Variant Details

DNA Sequence Variants

Gene	Amino Acid Change	Coding	Variant ID	Locus	Allele Frequency	Coverage	Transcript	Variant Effect
TP53	p.(R273C)	c.817C>T	COSM10659	chr17:7577121	10.88%	1470	NM_000546.5	missense

Shahanawaz Jiwani, MD, PhD, Laboratory Director | CLIA ID 21D2097127

Disclaimer: The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. The data version is 2023.06(006). The content of this report has not been evaluated or approved by FDA, EMA or other regulatory agencies.

Comments:
NA

Report Signed By

Name	Role	Date

Biomarker Descriptions

TP53 (tumor protein p53)

Background: The TP53 gene encodes the p53 tumor suppressor protein that binds to DNA and activates transcription in response to diverse cellular stresses to induce cell cycle arrest, apoptosis, or DNA repair. In unstressed cells, TP53 is kept inactive by targeted degradation via MDM2, a substrate recognition factor for ubiquitin-dependent proteolysis. Alterations in TP53 is required for oncogenesis as they result in loss of protein function and gain of transforming potential¹. Germline mutations in TP53 are the underlying cause of Li-Fraumeni syndrome, a complex hereditary cancer predisposition disorder associated with early-onset cancers^{2,3}.

Alterations and prevalence: TP53 is the most frequently mutated gene in the cancer genome with approximately half of all cancers experiencing TP53 mutations. Ovarian, head and neck, esophageal, and lung squamous cancers have particularly high TP53 mutation rates (60-90%)^{4,5,6,7,8,9}. Approximately two-thirds of TP53 mutations are missense mutations and several recurrent missense mutations are common including substitutions at codons R158, R175, Y220, R248, R273, and R282^{4,5}. Invariably, recurrent missense mutations in TP53 inactivate its ability to bind DNA and activate transcription of target genes^{10,11,12,13}.

Potential relevance: The small molecule p53 reactivator, PC14586, received a fast track designation (2020) by the FDA for advanced tumors harboring a TP53 Y220C mutation¹⁴. The FDA has granted fast track designation (2019) to the p53 reactivator, eprenetapopt,¹⁵ and breakthrough designation¹⁶ (2020) in combination with azacitidine or azacitidine and venetoclax for acute myeloid leukemia patients (AML) and myelodysplastic syndrome (MDS) harboring a TP53 mutation, respectively. In addition to investigational therapies aimed at restoring wild-type TP53 activity, compounds that induce synthetic lethality are also under clinical evaluation^{17,18}. TP53 mutations confer poor prognosis in multiple blood cancers including AML, MDS, myeloproliferative neoplasms (MPN), and chronic lymphocytic leukemia (CLL)^{19,20,21,22,23}. In mantle cell lymphoma, TP53 mutations are associated with poor prognosis when treated with conventional therapy including hematopoietic cell transplant²⁴. Mono- and bi-allelic mutations in TP53 confer unique characteristics in MDS, with multi-hit patients also experiencing associations with complex karyotype, few co-occurring mutations, and high-risk disease presentation as well as predicted death and leukemic transformation independent of the IPSS-R staging system²⁵.

Assay Information

Methodology, Scope, and Application:The OCAv3 next-generation sequencing (NGS) assay identifies more than 3000 annotated mutations of interest (MOIs) broadly categorized into 5 mutation types: single nucleotide variants (SNVs), small insertions/deletions (Indels), large (>3 bases) insertions/deletions (Large Indels), copy number variants (CNVs), and gene fusions. The assay utilizes the Thermo Fisher Scientific Oncomine® Comprehensive Assay v3.0 (OCAv3), a next-generation sequencing (NGS) assay that utilizes a multiplex polymerase chain reaction (PCR) with DNA and RNA extracted from formalin-fixed tissue for sequencing on the Ion S5 XL platform and analyzed by the current version of Torrent Suite Software and Ion Reporter. The OCAv3 NGS Assay currently can reliably identify the presence or absence of >3000 known mutations and polymorphisms in the 161 unique genes compared to the Human Reference Genome hg19, including the genes listed below. The OCAv3 assay is a laboratory developed test designed to find gene mutations within tumors (somatic mutations).

Analytical Sensitivity and Specificity:The OCAv3 NGS Assay has been determined to be suitably analytically sensitive and specific for the various types of abnormalities within its reportable range, demonstrating 97.18% sensitivity compared with orthogonal assay results

Shahanawaz Jiwani, MD, PhD, Laboratory Director | CLIA ID 21D2097127

and 100% specificity for the 5 mutation types reported. 99.99% or greater reproducibility has been demonstrated. All quality measures for this assay were within defined assay parameters.

Hereditary and Germline Mutations: This assay examines tumor tissue only and does not examine normal (non-tumor) tissue. Mutations detected by the assay may be present only in the tumor, or in every cell of the body (including non-tumor cells). This test also cannot tell whether a potential germline mutation causes or will cause a hereditary cancer syndrome. If the patient's history includes any one or combination of features suggestive of a possible hereditary cancer predisposition (such as, cancer arising at a young age, especially a cancer of a type unusual in younger patients; a personal history of multiple different types of cancers; or a significant cancer history among blood relatives, especially but not exclusively of the same type as the patient's) it is recommended that the physician arrange for the patient to meet with a genetic counselor and, if warranted, undergo the appropriate genetic test on normal (i.e. non-tumor) tissue (blood or cells brushed from the oral surface of the cheek) to check for a germline abnormality, regardless of the results of this research study.

Report Content: Thermo Fisher Scientific's Ion Torrent OncoPrint Reporter software was used in the generation of this report. Software was developed and designed internally by Thermo Fisher Scientific. The analysis was based on the current version of OncoPrint Reporter. The data presented here is from a curated knowledgebase of publicly available information, but may not be exhaustive. FDA information was sourced from www.fda.gov and is current as of this version. NCCN information was sourced from www.nccn.org and reflects its current version. Clinical Trials information reflects the current version. For the most up-to-date information regarding a particular trial, search www.clinicaltrials.gov by NCT ID or search local clinical trials authority website by local identifier listed in 'Other Identifiers.' Variants are reported according to HGVS nomenclature and classified following AMP/ASCO/CAP guidelines (Li et al. 2017). Based on the data sources selected, variants, therapies, and trials identified in this report are listed in order of potential clinical significance but not for predicted efficacy of the therapies.

Genes assayed for hotspot mutations:

AKT1, AKT2, AKT3, ALK, AR, ARAF, AXL, BRAF, BTK, CBL, CCND1, CDK4, CDK6, CHEK2, CSF1R, CTNNB1, DDR2, EGFR, ERBB2, ERBB3, ERBB4, ERCC2, ESR1, EZH2, FGFR1, FGFR2, FGFR3, FGFR4, FLT3, FOXL2, GATA2, GNA11, GNAQ, GNAS, H3F3A, HIST1H3B, HNF1A, HRAS, IDH1, IDH2, JAK1, JAK2, JAK3, KDR, KIT, KNSTRN, KRAS, MAGOH, MAP2K1, MAP2K2, MAP2K4, MAPK1, MAX, MDM4, MED12, MET, MTOR, MYC, MYCN, MYD88, NFE2L2, NRAS, NTRK1, NTRK2, NTRK3, PDGFRA, PDGFRB, PIK3CA, PIK3CB, PPP2R1A, PTPN11, RAC1, RAF1, RET, RHEB, RHOA, ROS1, SF3B1, SMAD4, SMO, SPOP, SRC, STAT3, TERT, TOP1, U2AF1, XPO1

Genes assayed with full exon coverage:

ARID1A, ATM, ATR, ATRX, BAP1, BRCA1, BRCA2, CDK12, CDKN1B, CDKN2A, CDKN2B, CHEK1, CREBBP, FANCA, FANCD2, FANCI, FBXW7, MLH1, MRE11, MSH2, MSH6, NBN, NF1, NF2, NOTCH1, NOTCH2, NOTCH3, PALB2, PIK3R1, PMS2, POLE, PTCH1, PTEN, RAD50, RAD51, RAD51B, RAD51C, RAD51D, RB1, RNF43, SETD2, SLX4, SMARCA4, SMARCB1, STK11, TP53, TSC1, TSC2

Genes assayed for copy number variations:

AKT1, AKT2, AKT3, ALK, AR, AXL, BRAF, CCND1, CCND2, CCND3, CCNE1, CDK2, CDK4, CDK6, EGFR, ERBB2, ESR1, FGF19, FGF3, FGFR1, FGFR2, FGFR3, FGFR4, FLT3, IGF1R, KIT, KRAS, MDM2, MDM4, MET, MYC, MYCL, MYCN, NTRK1, NTRK2, NTRK3, PDGFRA, PDGFRB, PIK3CA, PIK3CB, PPARG, RICTOR, TERT

Genes assayed for detection of fusions:

AKT2, ALK, AR, AXL, BRAF, BRCA1, BRCA2, CDKN2A, EGFR, ERBB2, ERBB4, ERG, ESR1, ETV1, ETV4, ETV5, FGFR1, FGFR2, FGFR3, FGR, FLT3, JAK2, KRAS, MDM4, MET, MYB, MYBL1, NF1, NOTCH1, NOTCH4, NRG1, NTRK1, NTRK2, NTRK3, NUTM1, PDGFRA, PDGFRB, PIK3CA, PPARG, PRKACA, PRKACB, PTEN, RAD51B, RAF1, RB1, RELA, RET, ROS1, RSPO2, RSPO3, TERT

DISCLAIMER

This assay is considered a Laboratory Developed Test (LDT). Its performance characteristics have been determined through extensive testing although it has not been cleared by the US Food and Drug Administration and such approval is not required for clinical implementation. Furthermore, any comments included in the report are strictly interpretive and the opinion of the reviewer. This laboratory is certified under the Clinical Laboratory Improvements Amendments (CLIA) for the performance of high-complexity testing for clinical purposes. The correlative data presented is a result of the curation of published data sources, but may not be exhaustive. The content of this report has not been evaluated or approved by the FDA or other regulatory agencies.

References

1. Muller et al. Mutant p53 in cancer: new functions and therapeutic opportunities. *Cancer Cell*. 2014 Mar 17;25(3):304-17. PMID: 24651012
2. Olivier et al. TP53 mutations in human cancers: origins, consequences, and clinical use. *Cold Spring Harb Perspect Biol*. 2010 Jan;2(1):a001008. PMID: 20182602
3. Guha et al. Inherited TP53 Mutations and the Li-Fraumeni Syndrome. *Cold Spring Harb Perspect Med*. 2017 Apr 3;7(4). PMID: 28270529
4. Cerami et al. The cBio cancer genomics portal: an open platform for exploring multidimensional cancer genomics data. *Cancer Discov*. 2012 May;2(5):401-4. PMID: 22588877
5. Weinstein et al. The Cancer Genome Atlas Pan-Cancer analysis project. *Nat. Genet*. 2013 Oct;45(10):1113-20. PMID: 24071849
6. Peter et al. Comprehensive genomic characterization of squamous cell lung cancers. *Nature*. 2012 Sep 27;489(7417):519-25. PMID: 22960745
7. Cancer Genome Atlas Network. Comprehensive genomic characterization of head and neck squamous cell carcinomas. *Nature*. 2015 Jan 29;517(7536):576-82. PMID: 25631445
8. Campbell et al. Distinct patterns of somatic genome alterations in lung adenocarcinomas and squamous cell carcinomas. *Nat. Genet*. 2016 Jun;48(6):607-16. PMID: 27158780
9. Cancer Genome Atlas Research Network. Integrated genomic characterization of oesophageal carcinoma. *Nature*. 2017 Jan 12;541(7636):169-175. doi: 10.1038/nature20805. Epub 2017 Jan 4. PMID: 28052061
10. Olivier et al. The IARC TP53 database: new online mutation analysis and recommendations to users. *Hum. Mutat*. 2002 Jun;19(6):607-14. PMID: 12007217
11. Rivlin et al. Mutations in the p53 Tumor Suppressor Gene: Important Milestones at the Various Steps of Tumorigenesis. *Genes Cancer*. 2011 Apr;2(4):466-74. PMID: 21779514
12. Petitjean et al. TP53 mutations in human cancers: functional selection and impact on cancer prognosis and outcomes. *Oncogene*. 2007 Apr 2;26(15):2157-65. PMID: 17401424
13. Soussi et al. Recommendations for analyzing and reporting TP53 gene variants in the high-throughput sequencing era. *Hum. Mutat*. 2014 Jun;35(6):766-78. PMID: 24729566
14. <https://www.globenewswire.com/news-release/2020/10/13/2107498/0/en/PMV-Pharma-Granted-FDA-Fast-Track-Designation-of-PC14586-for-the-Treatment-of-Advanced-Cancer-Patients-that-have-Tumors-with-a-p53-Y220C-Mutation.html>
15. <https://ir.aprea.com//news-releases/news-release-details/aprea-therapeutics-receives-fda-fast-track-designation>
16. <http://vp280.alertir.com/en/pressreleases/karolinska-development%27s-portfolio-company-aprea-therapeutics-receives-fda-breakthrough-therapy-designation-1769167>
17. Parrales et al. Targeting Oncogenic Mutant p53 for Cancer Therapy. *Front Oncol*. 2015 Dec 21;5:288. doi: 10.3389/fonc.2015.00288. eCollection 2015. PMID: 26732534
18. Zhao et al. Molecularly targeted therapies for p53-mutant cancers. *Cell. Mol. Life Sci*. 2017 Nov;74(22):4171-4187. PMID: 28643165
19. NCCN Guidelines® - NCCN-Acute Myeloid Leukemia [Version 3.2023]
20. Döhner et al. Diagnosis and management of AML in adults: 2022 recommendations from an international expert panel on behalf of the ELN. *Blood*. 2022 Sep 22;140(12):1345-1377. PMID: 35797463
21. NCCN Guidelines® - NCCN-Myelodysplastic Syndromes [Version 1.2023]
22. NCCN Guidelines® - NCCN-Myeloproliferative Neoplasms [Version 3.2022]
23. NCCN Guidelines® - NCCN-Chronic Lymphocytic Leukemia/Small Lymphocytic Lymphoma [Version 2.2023]
24. NCCN Guidelines® - NCCN-B-Cell Lymphomas [Version 2.2023]
25. Bernard et al. Implications of TP53 allelic state for genome stability, clinical presentation and outcomes in myelodysplastic syndromes. *Nat. Med*. 2020 Aug 3. PMID: 32747829